INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## TCHP: docetaxel, carboplatin, trastuzumab and pertuzumab

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**21 days**
See the active treatment phase

**Treatment stages**
**2**
Explained in the treatment reference

**Regimen source**
**NCCP 00722**
Version 2

Four medicines by drip on Day 1, every 21 days for six cycles. Delays or stopping earlier may be needed.

### Treatment stages

01 **Before surgery · six TCHP cycles**
21-day cycle

02 **Surgery and selected postoperative treatment**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

### Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

TCHP: docetaxel, carboplatin, trastuzumab and pertuzumab · National Cancer Control Programme (NCCP) 00722

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, chest symptoms, severe diarrhoea, dehydration, jaundice, unusual bleeding or sudden illness promptly. Do not wait for the next planned test.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## What each medicine does

Treat HER2-positive early breast cancer as part of a plan intended to reduce the risk of it returning.

TAXANE CHEMOTHERAPY

### Docetaxel



**Disrupts the cell’s division machinery**

Docetaxel stabilises tiny structures called microtubules. Cells need these to separate their chromosomes during division; disrupting the process can stop cancer cells multiplying. Healthy dividing cells are also affected.

Microtubules stabilised → Division disrupted → Cell growth affected

**What to watch for**

Take prescribed steroid premedication. Report fever, lasting numbness, swelling, sore nails or infusion symptoms.

PLATINUM CHEMOTHERAPY

## Carboplatin



### Damages deoxyribonucleic acid (the cell's genetic material) (DNA) inside cells

Carboplatin creates links in DNA, making it harder for cells to copy and repair their genetic instructions. Its dose depends on kidney function as well as the intended regimen.

Platinum enters cell → DNA links form → Copying is disrupted

**What to watch for**

Keep blood-test appointments. Fever, unusual bleeding, repeated vomiting or infusion symptoms need prompt advice.

HER2-TARGETED ANTIBODY

## Trastuzumab



### Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

**What to watch for**

Attend planned echoes. Report breathlessness, ankle swelling, rapid weight gain or infusion symptoms.

HER2-TARGETED ANTIBODY

### Pertuzumab



**Blocks HER2 pairing with other receptors**

Pertuzumab binds a different area of HER2 from trastuzumab. It limits receptor pairing, so the two antibodies act together at different points on the same growth pathway.

HER2 pairing → Pertuzumab blocks pairing → Growth signals reduced

**What to watch for**

Report diarrhoea early, especially with poor fluid intake. Keep cardiac checks and report infusion symptoms immediately.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Why this treatment?

TCHP treats higher-risk HER2-positive breast cancer before surgery using two chemotherapy medicines and two HER2-targeted antibodies. It avoids an anthracycline. Cancer stage, lymph nodes, fitness, heart and kidney function and expected side effects guide the choice. Surgery findings determine the next treatment.

**What could change the choice?**

Ask which cancer features, earlier treatments and health factors led to this recommendation, which alternatives were considered and when the decision will be reviewed.

## Your treatment and the next step

**Schedule**

All four medicines are given by infusion on Day 1 of a 21-day cycle, for six cycles before surgery. Surgery findings guide the postoperative treatment. There is no routine take-home chemotherapy pump.

**After this phase**

After surgery, the pathology helps select the next HER2 medicine. The plan usually aims to complete about one year of HER2 treatment from the first dose. Residual invasive disease may change the treatment. Blood tests and supportive medicines must follow the selected postoperative regimen.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

TCHP: docetaxel, carboplatin, trastuzumab and pertuzumab · NCCP 00722

## The phases and how they connect

### Treatment stages

01 **Before surgery · six TCHP cycles**
21-day cycle

02 **Surgery and selected postoperative treatment**
Individual plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

21-DAY CYCLE

### Before surgery · six TCHP cycles

**21-day cycle**

All four medicines are given by infusion on Day 1 of a 21-day cycle, for six cycles before surgery. Surgery findings guide the postoperative treatment. There is no routine take-home chemotherapy pump.

INDIVIDUAL PLAN

### Surgery and selected postoperative treatment

**Individual plan**

After surgery, the pathology helps select the next HER2 medicine. The plan usually aims to complete about one year of HER2 treatment from the first dose. Residual invasive disease may change the treatment. Blood tests and supportive medicines must follow the selected postoperative regimen.

Relative sequence, not equal lengths or booked dates. Your team confirms actual appointments and the active medicines.

BASELINE → TREATMENT → FOLLOW-UP

## Tests and milestones through treatment

Columns show sequence, not equal durations or appointments. A test is only complete when performed and reviewed. Echo timing and cancer-imaging timing are different. Symptoms can bring any check forward.

| Check | Before starting | Cycles 1–4 | Around cycle 5 / week 12 | Surgery / next phase | Ongoing / follow-up |
|---|---|---|---|---|---|
| **Blood tests** | Baseline full blood count (FBC), renal and liver profile | Before each cycle; additional dose checks as prescribed | Continue the required pre-dose checks | Use the selected active regimen's blood-test schedule | Investigate symptoms / persistent abnormalities |
| **Echo / cardiac assessment** | Baseline cardiac assessment | Earlier for symptoms / concern | Echo during neoadjuvant treatment, around cycle 5 | Surgery does not restart the echo clock | Agree follow-up for the active medicine and cardiac risk |
| **Cancer imaging / markers** | Baseline disease assessment | Clinical response; imaging if concern | Response review as planned; not tied to echo | Before surgery / pathology | Breast surveillance after early treatment |
| **Surgery / radiotherapy** | Agree indication and sequence | No automatic concurrent radiation | Coordinate with both teams | Local treatment as indicated | Review recovery and late effects |
| **Endocrine / bone health** | Check hormone receptors / fracture risk | Plan endocrine therapy if indicated | dual-energy X-ray absorptiometry (bone density) (DEXA) when relevant to bone-risk plan | Endocrine timing by the treating team | Individual DEXA / bone follow-up |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00722 v2 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · 2026

THE WHOLE TREATMENT JOURNEY

# TCHP: docetaxel, carboplatin, trastuzumab and pertuzumab: the whole pathway

## HER2-positive early breast cancer · before surgery

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | Cycles 1–4 | Around cycle 5 / week 12 | Surgery / next phase | Ongoing / follow-up |
|---|---|---|---|---|---|
| **Cancer medicines** NCCP | Confirm diagnosis and exact regimen | See the dose-day map: use the current regimen and active phase | Echo due; treatment review | Surgery/ pathology select postoperative medicine | Review benefit / tolerance and next plan |
| **Surgery / radiotherapy** EARLY | Agree indication and sequence | No automatic concurrent radiation | Coordinate with both teams | Local treatment as indicated | Review recovery and late effects |
| **Blood tests** NCCP | Baseline FBC, renal and liver profile | Before each cycle; additional dose checks as prescribed | Continue the required pre-dose checks | Use the selected active regimen's blood-test schedule | Investigate symptoms / persistent abnormalities |
| **Echo / cardiac checks** NCCP · ESC | Baseline cardiac assessment | Earlier for symptoms / concern | Echo during neoadjuvant treatment, around cycle 5 | Surgery does not restart the echo clock | Agree follow-up for the active medicine and cardiac risk |
| **Scans / tumour markers** EARLY | Baseline disease assessment | Clinical response; imaging if concern | Response review as planned; not tied to echo | Before surgery / pathology | Breast surveillance after early treatment |
| **Endocrine / DEXA** BONE | Check hormone receptors / fracture risk | Plan endocrine therapy if indicated | DEXA when relevant to bone-risk plan | Endocrine timing by the treating team | Individual DEXA / bone follow-up |

## What each check is for

1

### Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose.

Check safety before the next dose.

**Ask / act:** Agree the booking owner, result reviewer and action if overdue.

2

### Heart tests

Check heart pumping function before starting HER2 treatment. Repeat echo or multigated acquisition (heart pumping) scan (MUGA) every 12 weeks: around cycle 5, then cycle 9 if 21-day cycles stay on time. Earlier checks are needed for symptoms or abnormal findings. Count elapsed time from the first HER2 dose; changing phase or route does not restart the clock.

Monitor pumping function and relevant rhythm risk.

**Ask** / **act:** Agree the booking owner, result reviewer and action if overdue.

3



### Cancer imaging

Baseline breast and nodal imaging, with staging tests appropriate to the diagnosis. Review response during preoperative treatment and before surgery. After local treatment, arrange breast surveillance; routine computed tomography (CT), positron emission tomography (PET) or tumour-marker testing is not the usual screening plan for someone without symptoms.

Assess response or follow-up needs.

**Ask** / **act:** Agree the booking owner, result reviewer and action if overdue.

4

### Bone health

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

Assess fracture risk where relevant.

**Ask** / **act:** Agree the booking owner, result reviewer and action if overdue.

**At the next review, ask**

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Your practical treatment plan



STEP 1

### Before the first visit

Confirm blood tests, heart assessment, active medicines, route and the consent discussion.



STEP 2

### Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

### Infusion day

Trained staff check and access the line, give the prescribed premedicines and infusions, and monitor for reactions. Tell the nurse immediately about pain, burning, rash or breathlessness.

STEP 4

### Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

#### Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

#### PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

#### Visit time and transport

First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

## When scans, heart checks and bloods happen

Agree who books each test, who reviews the result and what happens if a check is overdue.

### Blood tests

FBC, kidney and liver profile at baseline and before every 21-day cycle. The team reviews the necessary results before a planned dose.

### Heart ultrasound and electrocardiogram (ECG)

Check heart pumping function before starting HER2 treatment. Repeat echo or MUGA every 12 weeks: around cycle 5, then cycle 9 if 21-day cycles stay on time. Earlier checks are needed for symptoms or abnormal findings. Count elapsed time from the first HER2 dose; changing phase or route does not restart the clock.

### Scans and tumour markers

Baseline breast and nodal imaging, with staging tests appropriate to the diagnosis. Review response during preoperative treatment and before surgery. After local treatment, arrange breast surveillance; routine CT, PET or tumour-marker testing is not the usual screening plan for someone without symptoms.

### Bone health and endocrine treatment

If the cancer is also hormone-receptor positive, endocrine treatment may form part of the wider plan. Bone-density testing is linked to aromatase inhibitors, ovarian suppression and fracture risk; it is not automatically required because a HER2 medicine is prescribed. Agree baseline DEXA where indicated and individual follow-up.

## How surgery and radiotherapy fit

Systemic treatment comes before surgery. Lumpectomy or mastectomy is chosen with the surgical team. Radiotherapy, if indicated, follows surgery and recovery; it may overlap postoperative HER2 treatment when both teams agree. It is not automatically concurrent with the chemotherapy phase.

SACTCHECK · CHAPTER 03

# Symptoms and supportive care

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## Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

| Treatment or check | Timing in this view |
|---|---|
| **All regimen infusions · Day 1** |  **Day 1** intravenous (IV) |
| **Blood counts / infection** |  **Days 1–21** Tests and symptoms guide review |
| **Energy and bowel symptoms** |  **Days 1–21** Timing varies; report changes early |
| **Numbness / tingling** |  **Days 1–21** Can accumulate with taxane treatment |
| **Heart symptoms** |  **Days 1–21** Any time; attend planned echoes |

**Planned treatment:** All four medicines are given by infusion on Day 1 of a 21-day cycle, for six cycles before surgery. Surgery findings guide the postoperative treatment. There is no routine take-home chemotherapy pump.

**Next phase:** After surgery, the pathology helps select the next HER2 medicine. The plan usually aims to complete about one year of HER2 treatment from the first dose. Residual invasive disease may change the treatment. Blood tests and supportive medicines must follow the selected postoperative regimen.

**Heart checks:** Check heart pumping function before starting HER2 treatment. Repeat echo or MUGA every 12 weeks: around cycle 5, then cycle 9 if 21-day cycles stay on time. Earlier checks are needed for symptoms or abnormal findings. Count elapsed time from the first HER2 dose; changing phase or route does not restart the clock.

**Why these bands stay broad:** Treatment markers show the planned schedule. Symptom bands are broad possibilities, not measured risk curves; serious symptoms can occur at any time. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00722 v2 · official regimen ↗

**There are no guaranteed "safe days".** Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override the timeline or diary.

### Between-treatment notes

### Day 1–21: The current treatment cycle

All four medicines are given by infusion on Day 1 of a 21-day cycle, for six cycles before surgery. Surgery findings guide the postoperative treatment. There is no routine take-home chemotherapy pump. Your team confirms treatment dates and checks. Serious symptoms need advice on any day.

## Support around the medicines

1 **The correct supportive prescription**

Follow the script for the active medicine, route and treatment day. Do not carry chemotherapy anti-sickness medicines automatically into an antibody-only phase.

2 **Keep heart checks on the plan**

An echo every 12 weeks is separate from blood tests or cancer imaging. Ask who books and reviews it.

3 **Plan for symptoms**

Keep your agreed contact numbers and instructions. New symptoms can need assessment before a routine appointment.

## Questions worth asking

### Who arranges my tests?

Ask the team who books each test, who checks the result and who to contact if an appointment is overdue. This map does not book appointments.

### What if I become unwell between visits?

Use the urgent contact plan for sudden illness, fever, chest symptoms, severe diarrhoea or poor fluid intake. Do not wait for a scheduled blood test or scan.

**Try explaining the plan back:** Which medicines are active now, which days do you receive or take them, and who is booking and reviewing your next tests?

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Day 1 · carboplatin combination

**Active medicines:** Docetaxel + Carboplatin + Trastuzumab + Pertuzumab

**Emetogenic risk:** High · carboplatin AUC ≥4

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>Reconcile chemotherapy premedication steroids |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Individual contraindications, renal function and duration review |
| Dexamethasone | 8 mg orally daily | Days 2–4<br>Carboplatin AUC ≥4 antiemetic row; reconcile total steroid exposure |

### Additional phase-specific support

- Docetaxel: dexamethasone 8 mg orally twice daily for 3 days starting the day before treatment; minimum 3 doses pre-treatment unless contraindicated. If oral doses were missed, any 20 mg intravenous (IV) replacement is a consultant decision.
- Reconcile docetaxel premedication with the antiemetic steroid course and interactions; issue one coherent steroid prescription, not duplicate courses.
- Consider primary granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) using the source and individual risk; product and timing follow the prescribed plan.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____

Prescriber / date: _____ Pharmacy review: _____

## Postoperative transition · confirm selected regimen

**Active medicines:** Selected postoperative medicine

**Emetogenic risk:** Confirm the selected postoperative regimen

The postoperative medicine is not selected by this template. Open its regimen before issuing supportive medicines.

### Additional phase-specific support

- No routine docetaxel or paclitaxel premedication or chemotherapy antiemetic course follows into antibody-only treatment.
- Phesgo or trastuzumab alone generally requires no routine antiemetic prophylaxis. Do not extend that statement automatically to an antibody-drug conjugate.
- Use the separately selected postoperative regimen for its blood tests, premedication, antiemetics and cardiac plan.
- Clinical template for individual review, not an issued prescription. Confirm allergy, pregnancy precautions, interactions and local policy.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Hair / scalp

**Docetaxel**

Hair loss is expected with docetaxel. Discuss scalp cooling where appropriate and hair / scalp support.

### 2 Mouth

**Chemotherapy**

Painful ulcers or difficulty eating and drinking need advice.

### 3 Breathing

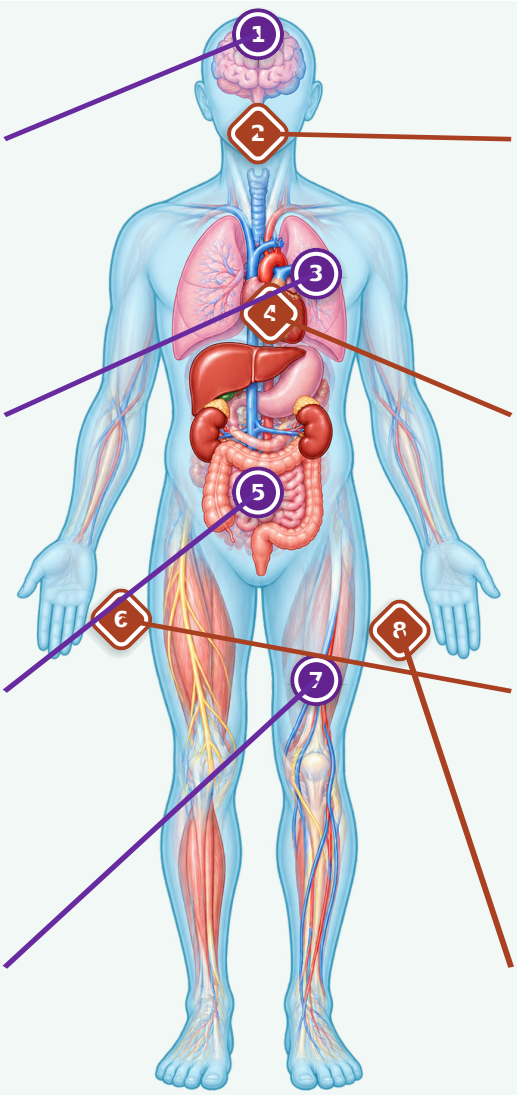
**HER2 treatment**

New cough or breathlessness needs prompt assessment.

### 4 Heart / circulation

**HER2 treatment**

Fainting, chest pain or severe breathing difficulty: urgent help. Keep scheduled cardiac checks.




### 5 Bowel

**Pertuzumab / docetaxel**

Report diarrhoea promptly, especially with poor intake, fever or dizziness.

### 6 Nerves / joints

**Docetaxel**

Report numbness, tingling or difficulty with buttons or walking before the next dose.

### 7 Blood / infection

**Chemotherapy and the whole regimen**

Fever, shivers or suddenly unwell: contact oncology immediately.

### 8 Skin / liver symptoms

**Treatment medicines**

Severe rash, blistering or yellow skin/eyes needs prompt advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, chest symptoms, severe diarrhoea, dehydration, jaundice, unusual bleeding or sudden illness promptly. Do not wait for the next planned test.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

# Sources and further information

TCHP: docetaxel, carboplatin, trastuzumab and pertuzumab · National Cancer Control Programme (NCCP) 00722

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00722 v2 · official regimen**
   https://healthservice.hse.ie/documents/6598/722_v2_TCHP.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
4. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
5. **ESMO metastatic breast cancer guideline · 2026**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
6. **EMA Herceptin product information**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
7. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
8. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
9. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **NCCP NCIS antiemetics v8 (2025), Table 1**
    https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
12. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
13. **EMA Perjeta product information**
    https://www.ema.europa.eu/en/documents/product-information/perjeta-epar-product-information_en.pdf